

How to Use Therascope

DOR guide · What each metric means · How to read your dashboard · 2026

therascope-insights.vercel.app

Therascope is your weekly performance dashboard. It shows your building's therapy metrics in real time based on the latest data uploaded by your regional team. Log in at **therascope-insights.vercel.app** using your facility DOR password. Data is uploaded three times per month — on the **10th, 20th, and last day of the month**.

Logging In

Go to	therascope-insights.vercel.app in any browser — works on desktop, tablet, or phone.
Select your building	Choose your facility from the dropdown. Each building has its own view.
Enter your password	Your DOR password is provided by your regional director. It is specific to your role — do not share it.
What you'll see	Your personal dashboard showing your building's current metrics, trends, and alerts for the latest data period.

The 4 Goal Metric Cards — Top Row

These are the metrics you are held accountable to. Green = at goal. Red = needs attention.

%	Productivity Goal: ≥84%
	Example display: 87.4% ✓ Meeting goal
	What it means: Billable treatment minutes divided by total paid on-site hours, expressed as a percentage. Shows how efficiently your therapists are converting their time into billed treatment.
	What to do if off goal: Review the schedule for gaps, cancellations, or excessive non-billable time. Check if any therapists have low individual productivity pulling the building average down.
\$	CPM Goal: ≤\$1.45
	Example display: \$1.48 Above \$1.45 target
	What it means: Cost Per Minute — total therapist labor cost divided by total treatment minutes billed. Captures both the cost of your staff AND how efficiently they bill. Travelers raise CPM significantly even at high productivity.
	What to do if off goal: Review staffing mix — how many travelers vs staff? Are travelers at high productivity? Escalate to your regional if CPM is consistently over target.
♥	Med B on Caseload Goal: ≥50%
	Example display: 40% (10 of 25 eligible)
	What it means: The percentage of your Medicare Part B eligible patients who are currently on active therapy caseload. Shows how well you are identifying and treating your Part B population.
	What to do if off goal: Review your census daily. Any Part B eligible patient not on caseload needs a clinical reason — refused, on hold, or recently discharged. If no reason, evaluate immediately.
■	Mode of Treatment Goal: >4%
	Example display: 8.3% ✓ Meeting 4% goal
	What it means: The percentage of total treatment time delivered as group or concurrent therapy. Shows efficient use of therapist time and scheduling. Must be clinically appropriate — not used just for productivity.

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What to do if off goal: Review scheduling. Are there patients who could appropriately be seen in group or concurrent? Refer to the Group & Concurrent guide in the DOR Playbook for clinical criteria.

The Lower Metric Cards — Second Row

These provide supporting context — not direct goal metrics, but important for understanding your department's performance.

Units / Visit (UPV)	What it shows: Average number of 15-minute billing units per patient visit. Target is generally 3.0+. Low UPV may indicate under-billing, short treatment sessions, or coding that doesn't capture all services provided. If low: Review treatment session lengths and CPT code utilization. Are therapists billing all codes appropriate to the session?
Med B Eligible	What it shows: The total number of patients in your building who are Medicare Part B payers. This is your potential caseload pool. Compare to "On Caseload" to see how many are being seen. If low: Cross-reference with your census daily. New admissions should be evaluated for Part B eligibility immediately.
Med B Units MTD	What it shows: Total Medicare Part B billing units generated so far this month (month-to-date). Shows the volume of Part B treatment being delivered across all disciplines. If low: If units are low relative to caseload, check for cancellations, low UPV, or patients not being scheduled consistently.
Medicare Rev MTD	What it shows: Estimated Medicare Part B revenue generated month-to-date based on units billed and the fee schedule. The projection label shows what your building is on pace to earn by month end at the current rate. If low: If revenue is behind projection, identify whether it's a caseload, UPV, or scheduling issue and address it immediately.

Week-over-Week Alerts

Automatically flags significant changes between this upload period and the previous one

The alerts section shows any metric that changed significantly compared to the previous week. A green alert means improvement. A red alert means decline. These are designed to catch problems early — before they show up as a bad month-end number.

↑ Productivity increased from 82% to 87%	→ Good — but verify it's from real caseload growth, not just fewer staff hours on-site.
↓ CPM increased from \$1.38 to \$1.61	→ Flag immediately. Check for new traveler, overtime, or productivity drop this week.
↓ Med B on caseload dropped from 52% to 38%	→ Likely a discharge or census change. Review who left caseload and whether new admissions were picked up.
↓ Mode of treatment dropped from 6.2% to 1.1%	→ Group/concurrent sessions may have been cancelled or not scheduled. Review the week's schedule.

Month-End Projections

Shows where your building is headed by end of month at the current pace

Each metric card that accumulates over time (revenue, units) shows a projection label: **"Proj: \$16.4k · May 31"**. This is calculated as: **current MTD value ÷ days elapsed × total days in month**. It tells you what you're on pace to earn if your current rate continues for the rest of the month.

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Projection is higher than expected	You're ahead of pace — good. Maintain caseload and scheduling through month end.
Projection is lower than expected	You're behind pace. Identify the gap — is it caseload, UPV, or cancelled visits? Every missed day is harder to recover late in the month.
Projection shows flat / same as current	Early in the month this is normal. Mid-month — check that data has been uploaded correctly.

The Resources Tab — Your DOR Playbook

The Resources tab is where your entire DOR Playbook lives. You'll find training guides, clinical protocols, assessment forms, and compliance references — all organized by category. You can view any document in your browser or download it.

Assessments	AMPAC, Joint Mobility, Am I Ready? Discharge Checklist, OT Home Evaluation Form
Training	Goal Writing, Productivity/CPM, Group & Concurrent, Discharge Criteria, Section GG, PCC guides
Protocols	Survey Prep, 8-Minute Rule, Functional Maintenance Documentation, CPT Clinical Guide
Forms	Administrative and clinical forms — updated as new forms are added

Your Therascope Review Routine — After Every Upload

1	Open Therascope	Data uploads on the 10th, 20th, and last day of the month. Check that the header date reflects the latest period before reviewing.
2	Read the alerts	Week-over-week alerts are at the bottom. Any red alert gets addressed before noon Monday.
3	Check the 4 goals	Productivity, CPM, Med B on caseload, Mode of treatment. Any metric off goal needs an action plan today.
4	Review projections	Are you on pace for the month? If revenue or units are behind, what is the recovery plan?
5	Check Med B eligible vs caseload	Is anyone eligible but not being seen? Review new admissions from the past week.
6	Open the DOR Weekly Self-Audit	Available in Resources → Protocols. Complete it every Monday — it takes 10 minutes and catches issues before they become problems.

Data is uploaded weekly by your regional team. Therascope shows MTD data — it reflects performance from the first of the month through the latest uploaded period. If something looks wrong or the data seems stale, contact your regional director. Do not make staffing or billing decisions based on data you believe is incorrect.